

August 13, 2026

South Carolina Medicaid (Healthy Connections / SCDHHS)
Provider Appeals Department

RE: Formal Appeal of Claim Denial — Renato Dare (MRN MUSC2558722), Claim MUSC-FFD5-1

Patient	Renato Dare (DOB 1976-06-14)
MRN	MUSC2558722
Member ID / Group	SCD346795729 / GRP-86928
Claim number	MUSC-FFD5-1
Date of service	2026-06-08
Procedure billed	CPT 99214 — Office/outpatient visit, established patient, moderate complexity
Primary diagnosis	J06.9 — Acute upper respiratory infection, unspecified
Denied amount	\$988.11
Denial code	CARC 11 / RARC M64 — The diagnosis is inconsistent with the procedure.
Appeal deadline	2026-08-19

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health East Cooper Medical Center submits this formal first-level appeal on behalf of patient Renato Dare (MRN: MUSC2558722, Member ID: SCD346795729) regarding the denial of Claim MUSC-FFD5-1 for services rendered on June 8, 2026, by rendering provider Sarah E. Whitmore, MD (NPI 1447382910). The claim was submitted on June 24, 2026, and denied on July 20, 2026, under CARC 11 ("The diagnosis is inconsistent with the procedure") and RARC M64 ("Missing/incomplete/invalid other diagnosis"), with the payer remark stating: "The diagnosis code reported does not support the procedure billed." The total denied amount is \$988.11. MUSC Health respectfully contends that this denial is not supported by the clinical record and requests full reversal and payment at the allowed amount of \$471.02.

CLINICAL BACKGROUND

Mr. Dare is a 50-year-old male with an extensive and active chronic disease burden documented in the medical record. His active conditions include type 2 diabetes with peripheral neuropathy, prediabetes, metabolic syndrome, hypertriglyceridemia, hyperglycemia, and anemia, all of which have been present and managed over a period of many years. He is an established patient currently maintained on extended-release metformin hydrochloride, a regimen that has been active since 2007. The June 8, 2026, encounter was billed as CPT 99214, an established patient office visit of moderate complexity, at Place of Service 22 (On Campus Outpatient Hospital). The primary diagnosis submitted was J06.9 (Acute upper respiratory infection, unspecified), with a secondary diagnosis of R69. While J06.9 was the presenting complaint driving the visit, the patient's active chronic conditions — including diabetes with neuropathy, metabolic syndrome, and hypertriglyceridemia — represent significant comorbidities that directly bear on the complexity of medical decision-making required at this encounter.

BASIS FOR APPEAL

The denial under CARC 11 and RARC M64 appears to rest on the premise that J06.9 alone is insufficient to support the level of service billed. MUSC Health agrees that J06.9 as a standalone acute diagnosis may not, in isolation, reflect the full clinical picture; however, the denial fails to account for the secondary diagnosis of R69 submitted on the claim, as well as the patient's well-documented active chronic conditions. CPT 99214 requires moderate complexity in medical decision-making, which encompasses the number and complexity of problems addressed, the amount of data reviewed, and the risk of complications. Mr. Dare's active diagnoses — type 2 diabetes with neuropathy, metabolic syndrome, hypertriglyceridemia, and anemia — represent multiple chronic conditions requiring ongoing management and directly elevate the complexity of any encounter, including one presenting with an acute upper respiratory complaint. An acute illness superimposed on poorly controlled or complex chronic disease is a recognized driver of higher-complexity evaluation and management services under current CPT guidelines. Furthermore, RARC M64 indicates that other diagnosis information was considered missing or invalid; the secondary diagnosis R69 was submitted and maps to active conditions in this patient's record, including hyperglycemia, metabolic syndrome, and hypertriglyceridemia. The clinical record supports the medical necessity and appropriate coding of the billed service.

REQUESTED ACTION

MUSC Health respectfully requests that South Carolina Medicaid (Healthy Connections / SCDHHS) reverse the denial of Claim MUSC-FFD5-1 in its entirety and reprocess the claim for payment at the established allowed amount of \$471.02. Should the Division of Appeals and Hearings require additional clinical documentation, operative notes, or a letter of medical necessity from Dr. Whitmore, MUSC Health will provide those materials promptly upon request. This appeal is submitted within the 30-day filing deadline of August 19, 2026.

Respectfully submitted,

Angela R. Whitfield, RN, BSN, CPC

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Enclosures

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record